

Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

- A. Neonatal respiratory distress syndrome (NRDS)
- B. Tracheoesophageal fistula
- C. Pneumonia
- D. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:

<think>
Analyzing the clinical presentation of the newborn, we notice the presence of cyanosis, tachypnea, subcostal and intercostal retractions, nasal flaring, and low oxygen saturation (88%). These symptoms suggest respiratory distress, but the key clues are the cyanosis that responds to oxygen administration and the absence of significant issues during nasogastric tube placement. This suggests that the issue is related to the lungs, rather than a cardiac anomaly or other conditions that would affect the entire body.

</think><answer> A. Neonatal respiratory distress syndrome (NRDS) </answer>

